

EVERY CHILD SURVIVES AND THRIVES - HEALTH

Country Programme of Cooperation
between the Government of Pakistan
and UNICEF 2018-2022



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2018-2022

RESULTS FOR CHILDREN

OVERVIEW

EVERY CHILD SURVIVES AND THRIVES - HEALTH

EVERY CHILD SURVIVES AND THRIVES - NUTRITION

EVERY CHILD LEARNS

EVERY CHILD IS PROTECTED FROM VIOLENCE AND
EXPLOITATION

EVERY CHILD LIVES IN A SAFE AND CLEAN
ENVIRONMENT

PROGRAMME EFFECTIVENESS

ADOLESCENTS

EARLY CHILDHOOD DEVELOPMENT

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OVERVIEW

UNICEF works together with the Government of Pakistan to ensure that the most disadvantaged newborns and children can benefit from integrated maternal, newborn and child health interventions, and healthy behaviours by 2022. UNICEF is prioritising newborn survival and immunization in 2018-2022 to ensure that every child survives and thrives.

Newborn mortality

The national neonatal mortality rate in Pakistan is 42 per 1,000 live births, while the Sustainable Development Goals (SDGs) aim for 12 per 1,000 live births.¹ Newborn girls and boys continue to die of conditions that could be managed within households, such as low birth weight, hypothermia and sepsis, as well as from conditions that require interventions in health facilities such as birth asphyxia and prematurity. Neonatal survival is further complicated by poorly performing cross-sectoral family care practices and interventions, including sub-optimal breastfeeding, hygiene and sanitation. Only 69 per cent of births are attended by skilled birth attendants, and the rate of exclusive breastfeeding for children younger than six months is low — only 38 per cent. The limited access to life-saving interventions is further complicated by huge disparities between provinces, rural and urban locations, income, and levels of education, particularly of mothers.

In December 2018, Pakistan's National Assembly passed a resolution in which it recognized the rights of every child born in Pakistan as well as mothers.² In particular, the Assembly resolved that “the State must ensure the provision of the best available early care and safe environment for all children. The House also recognized the importance of the first thousand days of every child's life, starting from conception, which is a unique period of opportunity for brain development and laying foundations of optimum health and future growth of children.”

Routine immunization

Universal and full immunization of children is crucial to reducing infant and child mortality and morbidity. While there is still a long way to go, the good news is that vaccination coverage in Pakistan has gradually increased from 35 per cent in 1990 to 66 per cent in 2018. It is encouraging that the percentage of children not receiving any of the six basic immunizations has decreased substantially since 1990, from 28 per cent to only 4 per cent in 2018.

UNICEF has continued to provide the Government of Pakistan with vital support to implement the national Expanded Program on Immunization (EPI), including planning, monitoring, procurement, as well as improving the governance, resourcing and supply chain of the immunization system. On the demand side, the work focuses on advocacy and social mobilization. It is also focused on introducing innovations to achieve wide-scale results. The concept of equity — reaching the hardest-to-reach children and the less privileged — underpins everything UNICEF does to ensure that children in Pakistan survive and thrive.

Polio

Pakistan is one of three remaining polio-endemic countries in the world, along with Afghanistan and Nigeria. Since the launch of Pakistan's Polio Eradication Programme in 1994, there has been a massive decline in polio cases from approximately 20,000 every year in the early 1990s to only eight cases in 2018. However, high-risk areas have remained reservoirs and the porous border with neighbouring Afghanistan requires continued concerted action.

An array of approaches and tools are implemented to help Pakistan reach the finish line of zero polio cases, with strong support from the government and the coordinated efforts of partners, including UNICEF. These approaches and tools are outlined in the National Emergency Action Plan of the Pakistan Polio Eradication Programme (NEAP). Given the frequent population movements between Pakistan and Afghanistan, the programme synchronizes and coordinates its operations with the Afghan Polio Eradication Programme.



Strengthened links between routine immunisation and polio

Links between routine immunization and polio have been strengthened under a federal taskforce, multiplying benefits for children. In 2018, UNICEF supported nearly 19,000 polio community health workers in the 11 districts with the lowest immunization coverage in 2018. They devoted a quarter of their time to mobilizing communities for routine immunization, identifying and referring children with no vaccinations against any disease. For example, polio workers supported Pakistan's October 2018 national measles campaign. During the five annual polio immunization campaigns, over 260,000 volunteer social mobilizers identified non-immunized children and referred them for routine vaccination. However, immense variations in successful referrals were seen, from 99 per cent in the Punjab province to as low as 4-16 per cent in Sindh where coordination between polio and EPI teams was weak in comparison. Nevertheless, innovations in immunization are collectively increasing routine immunization coverage, offering great promise as service delivery is strengthened.

EVERY CHILD SURVIVES



“Today, 74 in every 1,000 Pakistani children born alive die before their 5th birthday. This is why, from the Federal Ministry of Health and provincial authorities to the medical personnel, front-line workers and social mobilizers on the ground, everyone in Pakistan works to ensure that children no longer die of preventable disease. Our goal is to ensure that every child can survive and thrive. ”

Aida Girma
UNICEF Representative in Pakistan

GLOBAL TARGET

SDG 3: Sustainable Development Goal 3 aims to ensure healthy lives and well-being for all, including ending preventable deaths of newborns and children under 5 years of age. All countries aim to reduce neonatal mortality to at least as low as 12 per 1,000 live births and under-5 mortality to at least as low as 25 per 1,000 live births by 2030. This also means strengthening routine immunization with a target of 90 per cent of children 12-23 months old fully immunized by 2020.

UNICEF PAKISTAN TARGET

By 2022, the most disadvantaged newborns and children benefit from integrated newborn and child health interventions and healthy behaviour.

For every 1,000 babies that are born alive,
42 die as newborns and 62 die before age 1.



HOW UNICEF WILL HELP ACHIEVE RESULTS

UNICEF is working to ensure that the most at-risk newborns and children benefit from integrated newborn and child health interventions and healthy behaviours. We also support the Government of Pakistan to reach its target to provide integrated health services for newborns and children under the age of one in the most disadvantaged communities, in line with its national and global commitments.



THEORY OF CHANGE

If the policies, standards, guidelines and action plans on neonatal and child survival are planned and implemented with evidence-based data and based on verification from provinces, especially data from disadvantaged communities...



If political support and commitment are the driving forces behind setting accountability measures for improving health service delivery and demand creation...



If a health communication and promotion structure and system is operationalised to strengthen social mobilization and behaviour change communication to achieve our targets for health service delivery and demand creation...



If there is practical coordination and synergy between relevant sectors (Health, Polio, WASH and Nutrition) to improve access and utilization of basic services in disadvantaged communities...



If regular monitoring and evaluation is a standard practice within the health programme to check progress, periodically improve the programme design, and measure the interventions' impact for knowledge sharing, scale-up and replication...



...Then the most disadvantaged newborns and children will be benefiting from integrated newborn and child health interventions and healthy behaviours.

DATA PROFILE

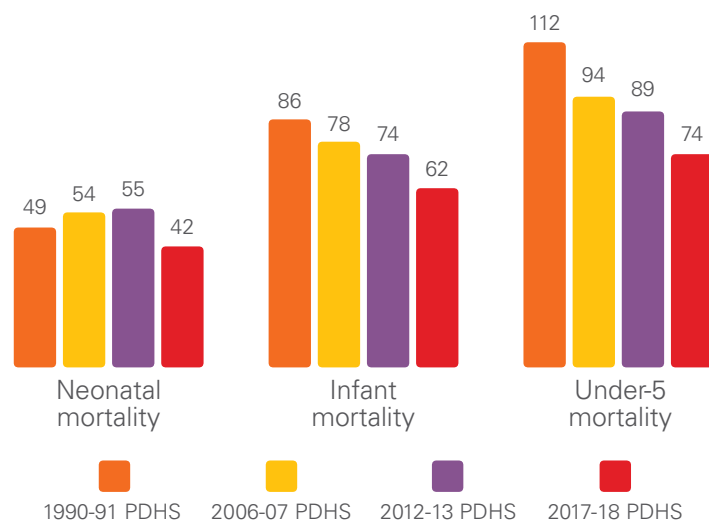
Newborn and child mortality

The overall under-5 mortality rate has declined steadily – from 112 deaths per 1,000 live births in 1984-1989 to 74 deaths per 1,000 live births in the most recent five-year period.³

Infant mortality under age 1 has decreased from 86 to 62 deaths per 1,000 live births over the same periods. Though neonatal mortality stagnated at 55 deaths per 1,000 live births for nearly a decade between 2006 and 2013, it has declined to 42 deaths per 1,000 live births in the most recent five-year period.⁴

Trends in childhood mortality

Deaths per 1,000



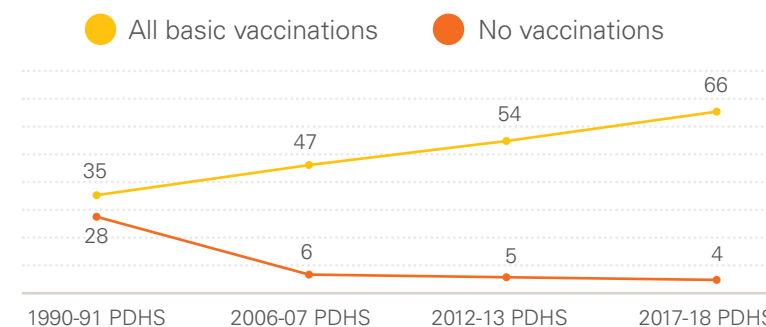
Note: Excludes Azad Jammu and Kashmir and Gilgit Baltistan.

Vaccination coverage

Full vaccination coverage has improved over the past five years from 54 per cent to 66 per cent, meaning that two-thirds of Pakistani children are now fully immunized. However, 4 per cent of children aged 12-23 months have not received any of the six basic vaccinations — a slight reduction in the past five years, but a remaining challenge.

Trends in childhood vaccinations

Percentage



Note: Excludes Azad Jammu and Kashmir and Gilgit Baltistan.

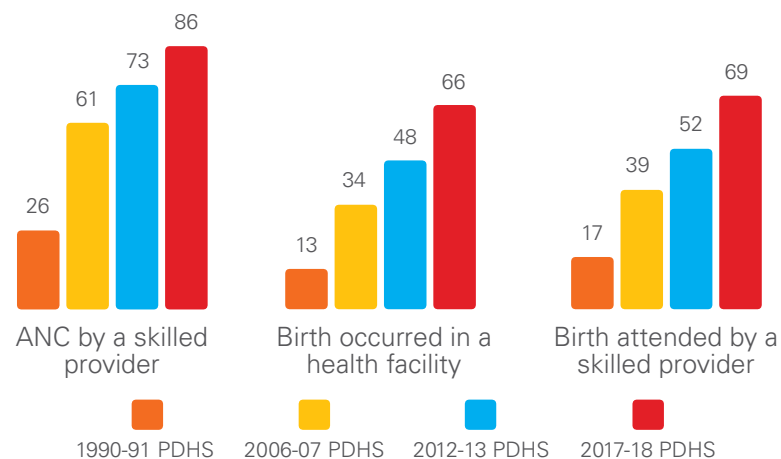


Maternal health care

Maternal health care is improving steadily. Currently 69 per cent of births are attended by skilled birth attendants.

Trends in maternal healthcare

Percentage



Note: Excludes Azad Jammu and Kashmir and Gilgit Baltistan.



ACCELERATING CHANGE IN PAKISTAN

Keeping every child alive: targeting quality care in health facilities and at home

Given that the Ministry of National Health Services & Regulation (MoNHSR) has an important role in the attainment of health goals, and given the focus on dignified maternal care in UNICEF's global Gender Action Plan, UNICEF supported the Ministry to develop and disseminate a strategic and accountability framework on quality of care at time of birth. These were disseminated to the provinces for discussion by the media, universities, health facilities and Provincial Healthcare Commissions in Punjab, Sindh and Khyber Pakhtunkhwa (KP). Provincial frameworks and plans were produced. The national "Reproductive Maternal Newborn Child Adolescent Health and Nutrition Strategy" and its costed plan were finalized in 2018 with technical support from UNICEF and WHO. Using this as a reference and incorporating new evidence and analytics, Punjab, Sindh and KP developed and endorsed newborn survival strategies and costed plans, including a Punjab policy on home visits for newborns based on a successful UNICEF-supported pilot.

With UNICEF support, the Governments of Sindh (all districts) and Punjab (five districts) strengthened health facilities to provide lifesaving services by localizing the "Global Action Plan for Pneumonia and Diarrhoea". In 2018, 2,421 health facilities were strengthened for management of diarrhoea and pneumonia using the integrated Management of Childhood Illness approach.⁵

National measles campaign

A 12-day national measles campaign was successfully conducted by the MoNHSR in October 2018 with UNICEF support, reaching 37 million children aged 9-59 months, exceeding initial targets. Despite challenges with funding, cascade training and quality assurance, the high coverage is partly attributable to stronger micro-planning and deployment of 163 people trained in the use of UNICEF's RapidPro



tool for monitoring and rapid course correction. UNICEF worked on the campaign in close partnership with provincial health departments, WHO and Gavi, the Vaccine Alliance.

Polio

During the five annual polio immunization campaigns in 2018, more than 260,000 volunteer social mobilizers identified un-immunized children and referred them for routine vaccinations. Under the Polio Eradication Initiative, UNICEF procured 349 million doses of oral and injectable polio vaccine, supporting scheduled and emergency response activities. Each of the five national campaigns vaccinated 38 million children, and five subnational campaigns vaccinated 20 million children. UNICEF also expanded its successful community-based vaccination approach to cover all 594 union councils in core polio reservoirs, deploying nearly 25,000 locally-recruited, trusted frontline workers (of whom 87 per cent were women). These reduced the proportion of never-vaccinated children and contributed to the drop from four to two in reservoirs of wild poliovirus cases.

EQUITY

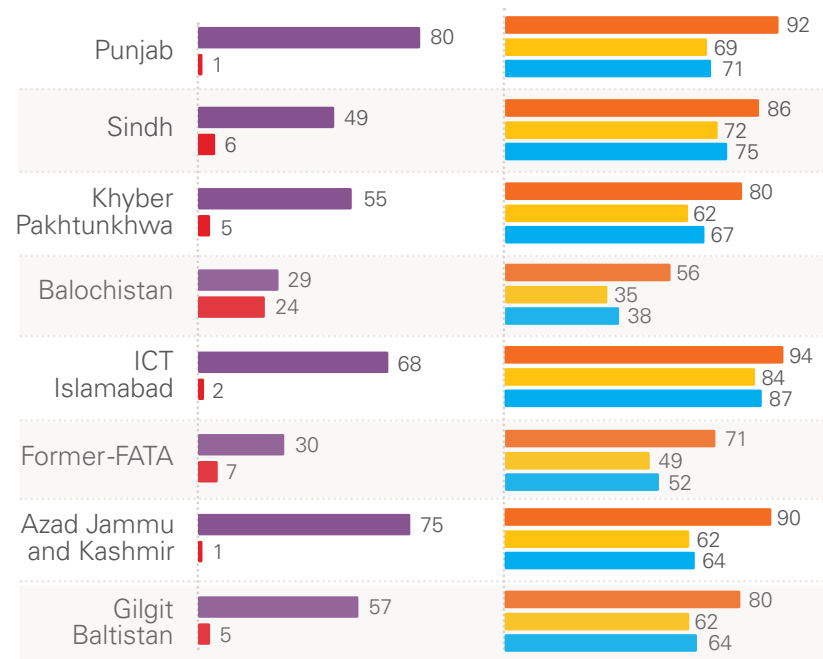
Immunization dropout rates are highest among the most marginalized and vulnerable Pakistani children, with huge disparities between provinces and according to whether the child lives in a rural or an urban setting. For instance, while full immunization coverage in the Punjab province stands at 80 per cent, it reaches only 29 per cent in the Balochistan province – less than one-third of children.⁶ Urban areas appear to do significantly better on full immunization at 71 per cent than rural areas at 63 per cent.⁷

Vaccination coverage

- All vaccines
- No vaccines

Maternal healthcare

- ANC by a skilled provider
- Birth occurred in a health facility
- Birth attended by a skilled provider



CHALLENGES

Enabling environment

- Insufficient prioritization by decision and policy makers on the importance of investing in primary and preventive health care, including against diarrhoea and pneumonia.
- Need to improve coordination among the Ministry of National Health Services, Regulation & Coordination and provincial governments.
- Need to improve coordination among partners at the federal level and in provinces.
- Need to strengthen governance and accountability mechanisms.
- Build capacity of the Health Management Information System (HMIS) at the federal level and in provinces with more data, disaggregated by gender and age.
- Advocacy for regulation of the private sector as it is a significant health services provider.



Supply — Availability of essential commodities and services

- There is weak integration of gender-responsive service delivery in health facilities and in communities.
- Supply chain management capacity and systems need to be strengthened to ensure adequate, consistent supply of essential health commodities.
- Storage capacities and reliable cold chain for vaccines and other essential medicines, such as pneumonia and diarrhoea, needs to be expanded and maintained.
- There are shortages of skilled health workers, Lady Health Workers (female community health workers), vaccinators and female frontline community health workers.
- Logistic support for outreach services and referrals needs to be improved.

Knowledge and demand for services

- Government capacity needs to be increased in the provinces and districts for social mobilization, behaviour change communication, and demand generation need.
- High transportation costs, out-of-pocket expenditures and fees for services in the private sector inhibit women from accessing health care services.
- Women's lack of mobility, very limited access to resources, and lack of time due to multiple responsibilities in daily life make it difficult for them to access and attain adequate healthcare services.
- Household decision-makers follow traditional practices such as no antenatal care and home delivery of babies.

Quality

- Execution of regular routines such as provinces' and districts' reviews, monitoring and supervision need to be improved.
- Minimum service delivery and quality of care standards for maternal newborn care and immunization need to be strongly established.



LOOKING FORWARD

Three deprivations are common to all provinces in Pakistan: (1) poor access to safe deliveries for newborn and maternal survival; (2) poor access to quality immunization services for protection against vaccine-preventable diseases; and (3) poor access to quality child health care services, particularly for diseases causing high mortality.

UNICEF's goal is that by 2022, the most at-risk mothers, newborns and children benefit from integrated health interventions and healthy behaviours. The Health programme collaborates with the Nutrition and WASH programmes to support Pakistan's National Health Vision for 2016-2025.

Ensuring access to quality immunization services:

UNICEF prioritizes the highest polio-risk districts, urban slums and union councils, targeting 800,000 children under the age of 1 year. We are supporting provincial health departments to deliver an integrated package encompassing immunization, neonatal services and prevention of pneumonia, diarrhoea and HIV. To reach all children, immunization programming will shift the focus to continuously missed children, including those in urban informal settlements. Priority is being given to supporting female frontline workers to reach these children. The goal is to achieve and sustain zero cases, leading to the interruption of virus transmission and to the certification of Pakistan as a polio-free country. UNICEF community-based vaccinators in the core reservoir zones will support the strengthening of immunization and healthcare systems.

Ensuring access to safe deliveries for newborns and addressing the high maternal mortality rate:

UNICEF is supporting provincial ministries of health to improve access to and quality of care at health facilities and in communities. The objectives are the improved survival of women during delivery, the prevention of stillbirths and neonatal survival rates. Enhanced accountabilities for quality of care among healthcare providers and



managers is promoted. Priority is given to high-impact healthcare practices and to working with Lady Health Workers. Civil society partnerships are used to influence positive behaviours among parents and other caregivers, while creating demand for skilled care. The Health programme also strengthens the capacity for planning, monitoring and quality assurance, co-creates evidence, and advocates for increased and sustained government budgets for maternal and newborn care and immunization.

Working across sectors:

The synergy between efforts to eradicate polio and increase routine immunization enables UNICEF to leverage and utilize existing platforms to serve previously unreached children. The Polio Eradication Initiative's large team of social mobilizers reaches children within high-risk communities at their doorstep, many in insecure areas or those characterized by low trust in vaccinations. UNICEF is leveraging their access to identify and target zero-dose children. As a result, previously unreached groups of children received routine vaccinations and the offer of the polio vaccine to communities was enhanced, reducing the impact of vaccination fatigue.

RESEARCH, KNOWLEDGE, EVALUATION

- **Situation Analysis of Children in Pakistan, 2017**
www.unicef.org/pakistan/reports/situation-analysis-children-pakistan
- **Pakistan Demographic and Health Survey 2017-2018**
 National Institute of Population Studies Islamabad, Pakistan:
<https://dhsprogram.com/pubs/pdf/PR109/PR109.pdf>
- **Capture the Moment: Early initiation of breastfeeding – the best start for every newborn**
 UNICEF, 2018: www.unicef.org/publications/index_102949.html
- **"4 Million Neonatal Deaths: When? Where? Why?"**
 Lawn, J.E., Cousens, S., Zupan, J. The Lancet. Neonatal Survival Series. p. 9-18. 2005: [www.thelancet.com/journals/lancet/article/PIIS0140-6736\(05\)71048-5/fulltext](http://www.thelancet.com/journals/lancet/article/PIIS0140-6736(05)71048-5/fulltext)



REAL LIVES, REAL CHANGE



Two years ago, Mafia Bibi, a mother of three, lost her baby when he was only five days old. This was the second time that she lost a newborn within the first few days of birth.

Losing her children one after another for unknown reasons was so heartbreaking to Mafia Bibi that she wanted to make sure she didn't conceive again anytime soon. She turned to her friend Razia, a community health worker — or Lady Health Worker as she is called in Pakistan — for advice on birth spacing.

"I have known Razia for several years now, but never really sought her help. Every time I had to deliver, I would travel to my parents' home, in another village," she says. "This time when I asked her for advice, she was very kind and guided me patiently. Two years passed by until I was pregnant again."

On Razia's recommendation, Mafia Bibi delivered her next child at a health facility in Bahawalnagar district, in the Punjab province, under the supervision of a doctor. Her previous children were all delivered at home without skilled birth attendants. Razia also counselled her on how to make sure her baby was as healthy as possible and she is now

exclusively breastfeeding her newborn, Mohammad Ramzan, who is one month old.

Pakistan has one of the highest newborn mortality rates in the world. This is due in part to a shortage of skilled birth attendants, as well as low rates of breastfeeding, immunization and antenatal care. Many residents in small villages live in poverty, without access to education or knowledge of good health and hygiene practices. Based on age-old traditions, most women rely on advice from the elderly for childbirth practices, which can pose a threat to the mother's and child's health.

To improve the survival and health care of newborns, UNICEF and partners trained all of Bahawalnagar's community health workers and their supervisors on how to improve newborn care at home. The health workers were given counselling cards with illustrations to help them guide the mothers in their communities. Through this project, UNICEF also procured thermometers, scales, portable projectors to show educational videos and acute respiratory infection timers that help the early diagnosis of pneumonia.

With new resources and enhanced capacity, community health workers are now contributing towards higher rates of births with skilled attendants, immunization, healthcare advice and quality care for newborns at home, and exclusive breastfeeding. All this is contributing to reduced rates of infant and child mortality.



Endnotes

- 1 National Institute of Population Studies. 2018. *Pakistan Demographic and Health Survey 2017-18*. Islamabad. <https://dhsprogram.com/pubs/pdf/PR109/PR109.pdf>.
- 2 National Assembly of Pakistan. *Resolution passed on December 21th, 2018*. www.na.gov.pk/en/resolution_detail.php?id=354.
- 3 The 5 years immediately preceding the 1990-91 Pakistan Demographic and Health Survey- PDHS. National Institute of Population Studies. 1991. *Pakistan Demographic and Health Survey 1990-91*. Islamabad. <https://dhsprogram.com/pubs/pdf/PR109/PR109.pdf>.
- 4 National Institute of Population Studies. 2007, 2013. *Pakistan Demographic and Health Survey (PDHS) 2006-07; 2012-13*. Government of Pakistan. Islamabad.
- 5 Because most child deaths occur at home, before reaching health facilities, preventing fatalities by improving child health through the community is at the core of the approach called Integrated Management of Childhood Illness (IMCI). This strategy has three main areas of focus: improving health worker skills, improving health systems and improving family and community practices (Community Integrated Management of Childhood Illness, C-IMCI).
- 6 *Pakistan Demographic and Health Survey 2017-18, op.cit.*
- 7 *Ibid.*

The maps in this publication are stylized and not to scale. They do not reflect a position by UNICEF on the legal status of any country or territory or the delimitation of any frontiers. The dotted line between Jammu and Kashmir represents approximately the Line of Control agreed upon by India and Pakistan. The final status of Jammu and Kashmir has not yet been agreed upon by the Parties.



for every child

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